



Armada Kalamunda Group

Complex Care Planning for Obstetric Patients

1. Purpose

The purpose of Complex Care planning is for the timely identification of women/babies at increased need of medical, midwifery, paediatric or allied health care and coordinates these services with a documented care plan. The care plan for the management of the woman's admission to hospital, including the discharge plan is available to all relevant staff when the woman is admitted for care.

2. Applicability

Complex care planning is appropriate for any woman with identified psychosocial risk factors including but not limited to mental illness, family and domestic violence, history of trauma and alcohol or substance misuse, +/- complex medical needs.

The complex care team meets fortnightly, where these cases are discussed and the need for enhanced multidisciplinary care or staff awareness is considered.

The management of the clinical needs of the woman and her unborn child remain the responsibility of the clinical team. When child protection concerns are identified a representative from the social work team will liaise with the DCPFS with regards to assessment, discharge and safety planning.

3. Guidelines

3.1 The Complex Care Team

- This multidisciplinary case conference (MCC) team consists of:
 - A representative from the Medical Obstetrics and Gynaecology Team
 - A Clinical Midwifery Manager
 - Antenatal Clinic Coordinator/Midwife and STAR midwife
 - Clinical Nurse Specialist Perinatal Mental Health - Maternity
 - A representative from the Social Work Department
 - A representative from the paediatric team

3.2 Functions of the Complex Care Team

- To optimise the management of women with increased care needs through an efficient and collaborative multi-disciplinary team approach.
- To identify those women and / or their family members who may pose a risk to AHS staff and other patients while AHS is providing Obstetric care.
- To ensure all relevant staff, inclusive of security services, are aware of potential risks to staff security from a patient and / or family member.
- To identify the need for an alert to the relevant hospital staff of high risk and complex patients who are likely to birth at Armadale Health Service.

- To identify women whose management may require an interagency planning meeting because of child protection / statutory action by the Department of Child Protection and Family Support when the baby is born.
- To identify women whose management may require interagency planning meeting because of serious mental illness.
- To ensure that the plan for the management of the woman and neonate, including the discharge plan is available to the relevant staff when the woman is admitted for care.

3.3 Referral Process

- Maternity staff – midwives and doctors - can refer to the Complex Care Team members through the e-referral system via the following health care professionals:
 - Clinical Nurse Specialist Perinatal Mental Health - Maternity
 - Social Worker (SW) if meets referral criteria does not require patient consent – document on referral if patient not consenting
 - Obstetrician
- Verbal/written referrals can be made to Antenatal Clinic Coordinator (ACC) for patients that don't meet criteria above but for whom there are significant concerns. Referral can then be discussed at the next meeting.

3.4 Identification, Registration and Discharge of Complex Care Patients

- Those women who are identified as needing complex care planning are entered on to the Complex Care Register. The register is kept current by the ACC. A brief description of the complex needs, and management plan are documented in the month of their expected due date.
- Complex care plan is documented on DMR under MDT tab
- When an at-risk woman is admitted, the shift coordinator will locate the management plan review any updates and relay the plan of care to the team via usual clinical handover processes.
- When women no longer meet the criteria for complex care, they will be resolved on the register by the ACC.

Criteria for Referrals

Complicated, serious or multiple issues including **but not limited to**:

- Serious Mental health disorders
- Family and Domestic Violence
- Special needs eg cognitive/learning difficulties
- Department for Child Protection and Family Services involvement
- Security implications
- Significant medical history or emerging issues
- Alcohol or Substance misuse and significant dependence
- Multiple DNA's

4. Roles and responsibilities

Role	Responsibility
Medical Obstetrics and Gynaecology Team member	A member of the Specialty Obstetrics Team at AHS. To be present at Complex Care MCC to provide advice on clinical/medical matters
Clinical Midwifery Manager	A member of the Midwifery management team – Associate Midwifery Manager – Ambulatory or Non-Ambulatory Services, or the Maternity Unit Manager. To be present at Complex Care MCC to provide advice on midwifery matters.
Antenatal Clinic Coordinator	A Clinical Midwife assigned to coordinate the Antenatal Clinic. Takes responsibility for the contemporaneous entry of notes into the Complex Care MCC register and the coordination of resources required for the meetings.
Clinical Nurse Specialist Mental Health - Maternity	The CNS undertakes assessments and provides management advice to the Obstetric and Midwifery Team for women who have a co-morbid mental illness. To be present at the MCC to provide advice on clients that have been referred for their assessment.
Social Work team member	Social workers are involved with identifying, supporting and managing vulnerable and high-risk families through care coordination, assessment, planning and intervention. To be present at the MCC to provide advice on clients that have been referred for their assessment and safety planning.
Paediatric team member	Paediatrician to formulate and document plan for neonates identified as at risk ie: substance/alcohol misuse, syphilis positive mother etc

5. Legislative context

The following documents are required to give effect to this policy

- Health Department of Western Australia (HDWA)
 - [Mandatory Clinical Handover Policy \(MP 0095\)](#)
- East Metropolitan Health Service (EMHS) Policy
 - [Patient Identification and Procedure Matching](#)
 - [Consent Policy EMHS: 5](#)

6. Supporting information

The following documents support the implementation of this policy:

- Armadale Kalamunda Group
 - [Antenatal Clinical Guideline](#)

- [Non-Standard Management Plans: Obstetrics and Gynaecology Procedure](#)

7. Compliance, monitoring and evaluation

Compliance against this guideline will be evaluated with monitoring of reported clinical incident data via Datix CIMS.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2
- Standard 5
- Standard 6

9. References

- Women's and Newborn Health Service. (2014). Planning for a Complex Care Needs Patient at KEMH.
<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/NMAHS/WNHS/WNHS.OG.ComplexCarePlanningfor.pdf>
- World Health Organisation. (1994). Lexicon of alcohol and drug terms. [9241544686_eng.pdf \(who.int\)](#)

10. Glossary

The following definitions are relevant to this policy.

Term	Definition
Alcohol or Substance misuse	Use of a substance for a purpose not consistent with legal or medical guidelines, as in the non-medical use of prescription medications. The term is preferred by some to abuse in the belief that it is less judgemental ¹ .
Multidisciplinary Case Conference	A Multidisciplinary Case Conference (MCC) is an appointment type where a patient (or carer) is not present and a meeting is arranged in advance and held concurrently between three or more healthcare providers to discuss a NAP to coordinate care. MCCs ensure that a patient's multidisciplinary care needs are met through a planned and coordinated approach.

11. Document owner

Enquiries relating to this guideline may be directed to:

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12. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	20 10 2025	20 10 2028	Full review, minor changes

13. Authorisation

This guideline has been authorised and issued by the Director of Clinical Services

Authorised by	Dr Tania Strickland - Director Clinical Services
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14. Appendix A: Quick Reference Flow Chart

